EMS CLINICAL CHECKLISTS — BATCH 4

Paramedic level with EMT delegation · CPAP CPAP / Bi-PAP (consolidated)

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

EDITION CHECK REQUIRED BEFORE ANY USE. The source file's cover says 'Last Update 07/01/2026'. Appendix A (PDF p.321) shows the 2026-2028 changes as Effective Nov. 1, 2026. 1001.1.h: after release of a new version, all previous versions are invalid. Confirm which edition is in force with agency clinical leadership (G-1).

SOURCE

Greater Miami Valley EMS Council standing orders, Regional Physician Advisory Board Region 3, file 2026protocol.pdf (328 pages). Page numbers = PDF viewer page index (1-328). Each protocol also prints its own 'Page x of y' footer; the printed footer is NOT used. Section numbers (e.g., 2008.2) are cited with every item.

CONTENTS OF THIS BATCH

Protocol	Title	Effective	Last modified	PDF pages used	Checkboxes	Not-specified lines
CPAP	CPAP / Bi-PAP (Non-Invasive Ventilation) — consolidated	June 1, 2021 (each source protocol)	1007: July 1, 2026; 4003 / 4013: May 16, 2026; 3008: Mar. 9, 2025; 1004: see Batch 1	PDF p.25 (1007), p.83 (3008), pp.121-122 (4003), p.143 (4013), p.19 (1004.3), p.181 (7001.3); Appendix A pp.321-323	15	5

HOW TO READ THE TAGS

Tag	Meaning (source)
MCP ORDER	Source marks the item with a diamond. Per 1001.5.d.iv, diamond items ALWAYS require a physician's order (PDF p.12).
{ } OPTION	Source brackets the item. Per 1001.5.d.v, bracketed items are at the option of the agency and its Medical Director (PDF p.12).
EMR+ / EMT+ / AEMT+ / PM	Lowest level at which the source's colored level table lists the item. Levels are cumulative: the table lists directives 'for the EMT and above', etc. (1001.6, PDF p.13).
A / P / G	Source markers: A = adult only, P = pediatric (pink), G = geriatric (1001.5.d.vi-viii, PDF p.12).

WHAT THE DIAMOND AND BRACE MARKS DO NOT TELL YOU

Your department's Medical Director decides which { } options are adopted (1001.4.d: nothing in the protocol may be used without specific pre-approval of the local Medical Director). 1001.4.e-f: the protocol is used as clinically indicated; not every standing order applies to every patient, and discretionary judgment is required.

STRUCTURE OF THIS FILE

Cover; one checklist in the order above (each starts with its own title bar and status banner and follows the seven-heading template); REQUIRES HUMAN VERIFICATION (full text of every gap and conflict); SOURCE-VERIFICATION TABLES (one row per checklist line). Use the companion HTML file for collapsible, tappable checklists.

AUDIT SUMMARY

Automated source audit at build: 17 checklist items in this file - every item has a section + PDF page citation and all key phrases were found in the cited pages (0 failures; whole-document audit 489 items, 0 failures); 19 numeric tokens compared against cited pages (only protocol-number / section pointers are not printed on the cited page). 5 lines are marked NOT SPECIFIED IN SOURCE. Provider levels were read from the rendered color bars. Human review is still required: this checklist is NOT marked fully verified.

Contrast check of the print palette (WCAG ratio): navy on white 11.48:1; body on white 17.06:1; teal on white 4.99:1; red on white 5.44:1; cream on navy 10.42:1; white on navy 11.48:1.

CONSOLIDATED REFERENCE (NO STAND-ALONE PROTOCOL) — CPAP / BI-PAP (NON-INVASIVE VENTILATION) — CONSOLIDATED

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION CPAP / Bi-PAP (Non-Invasive Ventilation) — consolidated	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Consolidated reference (no stand-alone protocol) — CPAP / Bi-PAP (Non-Invasive Ventilation) — consolidated	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 (each source protocol) · Last modified 1007: July 1, 2026; 4003 / 4013: May 16, 2026; 3008: Mar. 9, 2025; 1004: see Batch 1
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.25 (1007), p.83 (3008), pp.121-122 (4003), p.143 (4013), p.19 (1004.3), p.181 (7001.3); Appendix A pp.321-323) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

	ALL	SCOPE: the source contains NO CPAP / Bi-PAP procedure. 1007 mentions CPAP and {BiPAP} only in one Pearl. This checklist lists ONLY the lines that mention them in other protocols; everything else is marked NOT SPECIFIED IN SOURCE — VERIFY.	1007.1 Clinical Pearls · p.25
	INDICATIONS PRINTED IN THE SOURCE (ONE LINE PER PROTOCOL)		
☐	**EMT+**	ADULT pulmonary edema: CPAP (4013). *Condition / note: ADULT marker on the line; no pediatric line (see U-CPAP-3).*	4013.1 Tx Algorithm, EMT block · p.143
☐	**EMT+**	Bronchial constriction, any patient: CPAP (4003).	4003.1 Tx Algorithm, EMT block · p.121
☐	**EMT+**	Suspected smoke inhalation: consider CPAP (3008). *Condition / note: 'Consider' — no criteria printed (see U-CPAP-6).*	3008.4 Tx Algorithm, EMT block · p.83
☐	**PM** **{ } OPTION**	Persistent bronchial constriction: consider CPAP or {Bi-PAP} (4003).	4003.1 Tx Algorithm, Paramedic block · p.121
☐	**PM** **{ } OPTION**	Suspected smoke inhalation: if available consider {BiPAP} (3008).	3008.4 Tx Algorithm, Paramedic block · p.83
☐	**ALL**	DNR patient (after the DNR is initiated), if necessary for comfort or to relieve distress: may administer oxygen, CPAP or BiPAP. *Condition / note: Applies once the DNR is initiated; see 1004 in Batch 1 for DNR categories (see U-CPAP-8).*	1004.3.a.iv · p.19
	EQUIPMENT REQUIREMENT (DRUG BAG EXCHANGE PROGRAM)		
☐	**ALL**	To use an ALS/BLS or BLS drug bag in the pre-hospital emergency setting, the BLS provider list includes 'CPAP administration and management'. *Condition / note: The ALS provider list does not name CPAP (see U-CPAP-5).*	7001.3.l.i.4 · p.181

2 ASSESSMENT

☐	**EMR+**	Evaluate breath sounds; obtain pulse oximetry and capnography readings (pulmonary edema, 4013).	4013.1 Tx Algorithm, EMR block · p.143

CPAP / Bi-PAP patient selection, contraindications, mental-status and airway requirements, baseline vital-sign and SpO2 / EtCO2 targets: NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

	EMT LEVEL AND ABOVE		
☐	**EMT+**	If indicated, consider nebulized Albuterol 2.5 mg and Ipratropium 0.5 mg in line with CPAP (bronchial constriction, 4003). *Condition / note: 1007 still prints an MCP-order line for EMT nebulized medications (see U-CPAP-2).*	4003.1 Tx Algorithm, EMT block · p.121
☐	**ALL**	Where indicated, consider Albuterol 2.5 mg and Ipratropium 0.5 mg nebulized simultaneously inline with CPAP (4013 Pearls). *Condition / note: Level and indication not stated in the Pearl.*	4013.1 Clinical Pearls · p.143
☐	**EMT+** **MCP ORDER**	MCP ORDER (diamond, 1007): oxygen flow rate for nebulized medications should be 8-10 LPM; nebulized medication may be administered while ventilating with a BVM, preferably using two oxygen sources. *Condition / note: 1007 does not say whether this applies in line with CPAP (see U-CPAP-2).*	1007.1 Tx Algorithm, EMT block · p.25
	AEMT LEVEL AND ABOVE		
☐	**AEMT+**	ADULT, pulmonary edema: CPAP use is encouraged prior to the initiation of drug therapy. *Condition / note: 'Drug therapy' not defined (see U-CPAP-7).*	4013.1 Tx Algorithm, AEMT block · p.143

PARAMEDIC

☐	**PM { } OPTION**	Pulmonary edema: CPAP or {Bi-PAP} use is encouraged prior to the initiation of drug therapy.	4013.1 Tx Algorithm, Paramedic block · p.143
☐	**PM**	Pulmonary edema: consider need for possible early endotracheal intubation. *Condition / note: No link to CPAP failure is stated (see U-CPAP-1).*	4013.1 Tx Algorithm, Paramedic block · p.143

How to apply CPAP / Bi-PAP: device set-up, mask selection and fit, starting and maximum pressures, oxygen settings, titration, pediatric use, and how Bi-PAP differs: NOT SPECIFIED IN SOURCE — VERIFY (training / agency device instructions are not in the source).

EMT DELEGATION — from the source level tables

MAY	EMT: CPAP for adult pulmonary edema (4013 EMT block); CPAP for any bronchial-constricted patient, with nebulized Albuterol 2.5 mg + Ipratropium 0.5 mg in line if indicated (4003 EMT block); consider CPAP for suspected smoke inhalation (3008 EMT block). CPAP is NOT listed in any EMR block.
MCP ORDER	EMT, MCP ORDER (diamond in 1007): nebulized-medication oxygen flow 8-10 LPM and nebulizing while ventilating with a BVM; 1007 Consult says the EMT needs MCP ordered to administer nebulized medications. 4003 and Appendix A p.322 say the EMT requirement was removed — conflict, see U-CPAP-2.
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	Bi-PAP (Paramedic block, { } option); 'encouraged prior to drug therapy' sequencing (AEMT / Paramedic block); early endotracheal intubation (Paramedic block). Nothing in the source says who may set up or titrate CPAP beyond the level at which each line is printed.

4 REASSESSMENT / RESPONSE

	ALL	Pearl (1007): CPAP and {BiPAP} are forms of 'non-invasive ventilation', aggressive ventilatory support that avoids or eliminates the need for intubation.	1007.1 Clinical Pearls · p.25

Reassessment interval, response and failure criteria, when to stop or step up (e.g., to intubation), and monitoring while on CPAP / Bi-PAP: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**EMR+**	Call for transport (asthma / COPD, 4003; respiratory distress / pulmonary edema, 4013).	4003.1 Tx Algorithm, EMR block · p.121 4013.1 Tx Algorithm, EMR block · p.143

Destination, notification and handoff content for a patient on CPAP / Bi-PAP, and how the device is transferred or maintained during transport: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for CPAP / Bi-PAP use (device, settings, start / stop times, response, DNR comfort use): NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-CPAP-1	All sources (p.25)	No CPAP / Bi-PAP indications beyond the four lines above, no contraindications, no patient-selection criteria, no...
U-CPAP-2	1007.1 vs 4003.1 / Appendix A (p.25)	1007 (last modified July 1, 2026) Consult: 'The EMT needs MCP ordered to administer nebulized medications', with...
U-CPAP-3	4013.1 / 4003.1 / 3008.4 markers (p.143)	4013: the EMT and AEMT CPAP lines carry the ADULT marker; the Paramedic line (CPAP or {Bi-PAP}) carries none
U-CPAP-4	{ } agency option (p.121)	{Bi-PAP} / {BiPAP} is bracketed (agency / Medical Director option) in 4003, 4013, 3008 and the 1007 Pearl; 3008 adds...
U-CPAP-5	7001.3.l (p.181)	The BLS provider equipment list names 'CPAP administration and management'; the ALS provider list printed does not...
U-CPAP-6	3008.4 (p.83)	'Consider CPAP for suspected smoke inhalation' (EMT block) has no criteria, and no stated link to the cyanide...
U-CPAP-7	4013.1 / 4003.1 (p.143)	'CPAP use is encouraged prior to the initiation of drug therapy' — 'drug therapy' is not defined (4013 lists...
U-CPAP-8	1004.3.a.iv (p.19)	After a DNR is initiated, CPAP or BiPAP may be given 'if necessary, for comfort or to relieve distress'
U-CPAP-9	Editions (p.25)	1007 shows 'Last modified July 1, 2026'; Appendix A is dated June 6, 2026 (effective Nov

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

REQUIRES HUMAN VERIFICATION

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Global issues (all protocols in this file)

ID	Topic	Issue	Clarification needed
G-1	Edition / effective date	Cover (PDF p.1): 'Last Update 07/01/2026'. Appendix A (PDF p.321): '2026-2028 Protocol Changes', Effective Nov. 1, 2026. Each protocol prints 'Effective: June 1, 2021' and its own 'Last modified'. 1001.1.h: after release of a new version, all previous versions are invalid. Today is Oct. 30, 2026.	Confirm with agency clinical leadership which edition is in force today and on Nov. 1, 2026 before any field use.
G-2	Level mapping method	Provider levels come from the colored side bars of each Clinical Management Table (grey EMR, orange EMT, green AEMT, blue Paramedic). Text extraction cannot carry that information, so levels were read from rendered page images and cross-checked against Appendix A and the monographs.	Reviewer should spot-check level tags against the printed color bars.
G-3	Cross-referenced protocols not reproduced	Referenced but NOT built or reviewed in Batches 1-4: 1005 (only quoted lines used), 1007 (only the CPAP / nebulizer lines used; oxygen rates and intubation criteria NOT reproduced), 1008/1009 (airway), 1012 (IO), 1014 (only quoted lines), 3001, 3015, 3016, 3017, 3018, 3020, 4005, 4008, 4011, 4016 (shock), 8035 (norepinephrine), 8036 (fluids), 7012/7013, 3019/3021. Batch 2 built 2010, 2011, 3002, 4002, 4017; Batch 3 built 2001, 4003, 4012, 4013, 4014; Batch 4 is the consolidated CPAP / Bi-PAP checklist.	Tell me which to add in later batches.
G-4	Agency options and Medical Director approval	Items in { } are agency/Medical Director options (1001.5.d.v) and 1001.4.d says nothing may be used without Medical Director pre-approval. This checklist cannot know which options your department has adopted. 1001.4.e-f: protocol is used as clinically indicated; not every standing order applies to every patient.	Agency Medical Director to mark adopted options.
G-5	Monograph level columns	Drug monographs (8000 series) print level tabs as column headers; the text extraction does not show which column each row sits in. Medication level statements in the checklist rely on the protocol tables, with monographs used for doses, contraindications and Medical Control lines.	Reviewer to confirm any level-specific monograph statement.
G-6	Status	This checklist has NOT been reviewed or approved by agency clinical leadership and does not authorize care. Reviewer sign-off lines in Section 7 are intentionally blank.	Complete the review sign-off.
G-7	Monograph cross-check scope and numbering	Batch 2 doses were compared with monographs 8001, 8002, 8003, 8005, 8006/8007, 8013, 8017, 8018, 8019, 8023, 8024, 8026, 8029, 8030, 8031 and 8042 (Medical Control lines, dosing, contraindications). Footer / header numbers are inconsistent in the source: the Calcium Gluconate page header reads 8007 but its footer reads '8006 - Calcium Gluconate'; the Norepinephrine page header reads 8035 but its footer reads '8034 - Norepinephrine'; Nitroglycerin is 8034. Monograph level columns were not read (G-5).	Reviewer: confirm drug and number mapping; no clinical content was inferred from the numbering.

CPAP CPAP / Bi-PAP (Non-Invasive Ventilation) — consolidated

ID	Source location	Issue	Clarification needed
U-CPAP-1	All sources (p.25)	No CPAP / Bi-PAP indications beyond the four lines above, no contraindications, no patient-selection criteria, no device settings or pressures, no pediatric guidance, no monitoring, no failure criteria and no step-up pathway are printed anywhere in the source. 1007 (Basic Airway Maintenance) contains only a Pearl; 7001.3.l requires 'CPAP administration and management' equipment for the BLS provider but gives no content. Early intubation (4013) is not tied to CPAP failure.	Medical Director / agency: supply the CPAP / Bi-PAP procedure, contraindications and settings; do not use this checklist as a procedure.
U-CPAP-2	1007.1 vs 4003.1 / Appendix A (p.25)	1007 (last modified July 1, 2026) Consult: 'The EMT needs MCP ordered to administer nebulized medications', with diamonds on 'Oxygen flow rate for nebulized medications should be 8-10 LPM' and on nebulizing while ventilating with a BVM. 4003 EMT block (May 16, 2026) prints no MCP order for the same nebulized Albuterol / Ipratropium, and Appendix A (p.322) says the 4003 EMT MCP requirement was removed. 4002 still prints the order (U-4002-1). The newer-dated page (1007) still has the requirement; the nebulized-in-line-with-CPAP EMT treatment depends on which text governs.	Medical Director: which text governs; correct 1007 / 4002.
U-CPAP-3	4013.1 / 4003.1 / 3008.4 markers (p.143)	4013: the EMT and AEMT CPAP lines carry the ADULT marker; the Paramedic line (CPAP or {Bi-PAP}) carries none. 4003 and 3008 CPAP lines carry no age marker. 4013 Pediatric Considerations: None. Whether CPAP may be used in pediatric patients (and from what age or size) is not stated; 1005.4 defines pediatric as less than 16 years.	Medical Director: pediatric CPAP rule.
U-CPAP-4	{ } agency option (p.121)	{Bi-PAP} / {BiPAP} is bracketed (agency / Medical Director option) in 4003, 4013, 3008 and the 1007 Pearl; 3008 adds 'If available'. 1004.3.a.iv lists 'BiPAP' without brackets. 1001.4.d: nothing is used without local Medical Director pre-approval.	Agency: is Bi-PAP adopted and approved?
U-CPAP-5	7001.3.l (p.181)	The BLS provider equipment list names 'CPAP administration and management'; the ALS provider list printed does not name CPAP, although AEMT and Paramedic blocks reference it. The line preceding the lists says 'unless otherwise noted'. Whether an ALS unit must carry CPAP is not stated.	Confirm with the Drug Bag Exchange Program.
U-CPAP-6	3008.4 (p.83)	'Consider CPAP for suspected smoke inhalation' (EMT block) has no criteria, and no stated link to the cyanide treatment on that page (3008 is not built in Batches 1-4). The Paramedic {BiPAP} line is 'If available'.	Medical Director: criteria for smoke inhalation.
U-CPAP-7	4013.1 / 4003.1 (p.143)	'CPAP use is encouraged prior to the initiation of drug therapy' — 'drug therapy' is not defined (4013 lists nitroglycerin at the AEMT level). 4003 does not say whether CPAP precedes epinephrine, Solu-Medrol or magnesium. Combined use of CPAP with nitroglycerin / blood pressure limits is not discussed.	Medical Director clarification.
U-CPAP-8	1004.3.a.iv (p.19)	After a DNR is initiated, CPAP or BiPAP may be given 'if necessary, for comfort or to relieve distress'. Relationship to the DNR categories (1004.2, Batch 1) and to the {BiPAP} adoption question is not stated.	Confirm against the 1004 checklist.
U-CPAP-9	Editions (p.25)	1007 shows 'Last modified July 1, 2026'; Appendix A is dated June 6, 2026 (effective Nov. 1, 2026) and lists only a 1007 Pearl addition ('Added emphasis that CPAP and {BiPAP} are forms of Non-invasive Ventilation'). Appendix A lists 4003 / 4013 inline-nebulizer additions. See G-1 for the edition question.	Confirm edition in force.

SOURCE-VERIFICATION TABLES

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One row per checklist line. 'Traced' = the cited section and PDF page were located and the item's key phrases and numbers were machine-matched to that page; levels were read from the rendered color bars. Human review is still pending. Level column: lowest level at which the source lists the item (cumulative).

CPAP CPAP / Bi-PAP (Non-Invasive Ventilation) — consolidated — PDF p.25 (1007), p.83 (3008), pp.121-122 (4003), p.143 (4013), p.19 (1004.3), p.181 (7001.3); Appendix A pp.321-323

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
CPAP-1.01	SCOPE: the source contains NO CPAP / Bi-PAP procedure. 1007...	1007.1 Clinical Pearls (p.25)	ALL	-	Traced; review pending
CPAP-1.02	ADULT pulmonary edema: CPAP (4013).	4013.1 Tx Algorithm, EMT block (p.143)	EMT+	ADULT marker on the line; no pediatric line (see U-CPAP-3).	Traced; review pending
CPAP-1.03	Bronchial constriction, any patient: CPAP (4003).	4003.1 Tx Algorithm, EMT block (p.121)	EMT+	-	Traced; review pending
CPAP-1.04	Suspected smoke inhalation: consider CPAP (3008).	3008.4 Tx Algorithm, EMT block (p.83)	EMT+	'Consider' — no criteria printed (see U-CPAP-6).	Traced; review pending
CPAP-1.05	Persistent bronchial constriction: consider CPAP or {Bi-PAP}...	4003.1 Tx Algorithm, Paramedic block (p.121)	PM	-	Traced; review pending
CPAP-1.06	Suspected smoke inhalation: if available consider {BiPAP}...	3008.4 Tx Algorithm, Paramedic block (p.83)	PM	-	Traced; review pending
CPAP-1.07	DNR patient (after the DNR is initiated), if necessary for...	1004.3.a.iv (p.19)	ALL	Applies once the DNR is initiated; see 1004 in Batch 1 for DNR categories (see U-CPAP-8).	Traced; review pending
CPAP-1.08	To use an ALS/BLS or BLS drug bag in the pre-hospital emergency...	7001.3.l.i.4 (p.181)	ALL	The ALS provider list does not name CPAP (see U-CPAP-5).	Traced; review pending
CPAP-2.01	Evaluate breath sounds; obtain pulse oximetry and capnography...	4013.1 Tx Algorithm, EMR block (p.143)	EMR+	-	Traced; review pending
CPAP-2.02	CPAP / Bi-PAP patient selection, contraindications...	-		-	NOT SPECIFIED — VERIFY
CPAP-3.01	If indicated, consider nebulized Albuterol 2.5 mg and...	4003.1 Tx Algorithm, EMT block (p.121)	EMT+	1007 still prints an MCP-order line for EMT nebulized medications (see U-CPAP-2).	Traced; review pending
CPAP-3.02	Where indicated, consider Albuterol 2.5 mg and Ipratropium 0.5...	4013.1 Clinical Pearls (p.143)	ALL	Level and indication not stated in the Pearl.	Traced; review pending
CPAP-3.03	MCP ORDER (diamond, 1007): oxygen flow rate for nebulized...	1007.1 Tx Algorithm, EMT block (p.25)	EMT+	1007 does not say whether this applies in line with CPAP (see U-CPAP-2).	Traced; review pending
CPAP-3.04	ADULT, pulmonary edema: CPAP use is encouraged prior to the...	4013.1 Tx Algorithm, AEMT block (p.143)	AEMT +	'Drug therapy' not defined (see U-CPAP-7).	Traced; review pending
CPAP-3.05	Pulmonary edema: CPAP or {Bi-PAP} use is encouraged prior to...	4013.1 Tx Algorithm, Paramedic block (p.143)	PM	-	Traced; review pending
CPAP-3.06	Pulmonary edema: consider need for possible early endotracheal...	4013.1 Tx Algorithm, Paramedic block (p.143)	PM	No link to CPAP failure is stated (see U-CPAP-1).	Traced; review pending
CPAP-3.07	How to apply CPAP / Bi-PAP: device set-up, mask selection and...	-		-	NOT SPECIFIED — VERIFY
CPAP-4.01	Pearl (1007): CPAP and {BiPAP} are forms of 'non-invasive...	1007.1 Clinical Pearls (p.25)	ALL	-	Traced; review pending
CPAP-4.02	Reassessment interval, response and failure criteria, when to...	-		-	NOT SPECIFIED — VERIFY
CPAP-5.01	Call for transport (asthma / COPD, 4003; respiratory distress /...	4003.1 Tx Algorithm, EMR block (p.121); 4013.1 Tx Algorithm, EMR block (p.143)	EMR+	-	Traced; review pending
CPAP-5.02	Destination, notification and handoff content for a patient on...	-		-	NOT SPECIFIED — VERIFY
CPAP-6.01	Documentation / closeout elements for CPAP / Bi-PAP use...	-		-	NOT SPECIFIED — VERIFY